

**Laboratory Investigation Report**

Patient Name	: Sana Khan	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 37 Y 0 M 0 D /F	OP/IP No	: OP/101330
Max ID/Mobile	: 452256/8752005200	Collection Date/Time	: 10/Sep/2021 04:36PM
Lab ID	: 1266092100242	Receiving Date	: 11/Sep/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 11/Sep/2021
Passport No.	:		

Blood Bank**Blood Grouping and RH Factor*, EDTA**

Date	10/Sep/2021 04:36PM	Unit	Bio Ref Interval
Blood Group	O		
Haemagglutination	NEGATIVE		

Kindly correlate with clinical findings

***** End Of Report *****Dr. Sangeeta Pathak , DIHBT
Head-Transfusion MedSIN No: B2B899494, Test Performed at : 1103 - Max Hospital Saket (East Block), 1, 2, Press Enclave Marg, Saket Institutional Area, Saket, New Delhi
Max Lab - A Division of Max Healthcare Institute Ltd.

Booked at: Max Super Specialty Hospital, Saket (West Block), 2nd Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, 1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.

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CIN No.: L72200MH2001PLC322854
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Clinical Biochemistry

KFT Profile with Calcium,Uric Acid*, Serum

Date	10/Sep/2021 04:43PM	Unit	Bio Ref Interval
Urea Enzymatic Rate (Urease)	22.5	mg/dL	17.12 - 55.64
Creatinine Rate-Jaffe	0.92	mg/dl	0.61 - 1.24
eGFR MDRD	68.69	ml/min/1.73 m ²	
Uric Acid Enzymatic Trinder	4.36	mg/dl	2.6 - 8.0
Calcium (Total) ISE Indirect	9.41	mg/dl	8.9 - 10.3
Sodium ISE Indirect	138.9	mmol/L	136 - 144
Potassium ISE Indirect	4.31	mmol/L	3.6 - 5.1
Chloride ISE Indirect	102.1	mmol/l	101-111
Bicarbonate ISE Indirect	22.1	mmol/l	22-32

Interpretation Ref. Range

eGFR - Estimated Glomerular Filtration Rate is calculated by MDRD equation which is most accurate for GFRs $\leq$ 60ml / min /1.73 m².MDRD equation is used for adult population only.

<60ml / min / 1.73 m² - Chronic Kidney Disease

<15 ml / min /1.73 m² - Kidney failure



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Clinical Biochemistry

Liver Function Test Profile,Serum*

Date	10/Sep/2021 04:43PM	Unit	Bio Ref Interval
Total Protein	6.94	g/dl	6.5 - 8.1
Biuret			
Albumin	4.6	g/dl	3.5 - 5.0
BCP			
Globulin	2.3	g/dl	2.3 - 3.5
Calculated			
A.G. ratio	2.0		1.2 - 1.5
Calculated			
Bilirubin (Total)	0.60	mg/dl	0.3 - 1.2
Diazo			
Bilirubin (Direct)	0.26	mg/dl	0.1 - 0.5
Diazo			
Bilirubin (Indirect)	0.34	mg/dL	0.1 - 1.0
Calculated			
SGOT- Aspartate Transaminase (AST)	34.31	U/L	10 -35
UV without P5P			
SGPT- Alanine Transaminase (ALT)	28.83	U/L	< 40
Kinetic Rate using LDH			
Alkaline Phosphatase	132.84	U/L	32 - 91
PNP AMP Buffer			
GGTP (Gamma GT), Serum	13.0	U/L	7 - 50
Enzymatic Rate			



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Clinical Biochemistry
Blood Sugar (Random), Fluoride Plasma*

Date	10/Sep/2021 04:40PM	Unit	Bio Ref Interval
Random Glucose UV-Hexokinase	83.51	mg/dl	74 - 140

Interpretation A random plasma glucose of > 200 mg/dl with symptoms of diabetes is diagnostic. A confirmatory fasting plasma Glucose (FPG) test or OGTT should be completed on a different day if the clinical condition of the patient permits.

Kindly correlate with clinical findings

*** End Of Report ***

Mrinalini
Dr. Mrinalini Priyadarshini
 DCP, DNB (Path.)
 Consultant Pathologist





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Clinical Pathology

Urine Routine And Microscopy*

Date	10/Sep/2021 04:43PM	Unit	Bio Ref Interval
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Macroscopy

Reflectance photometry

Colour	Pale Yellow		Pale Yellow
PH	6.0	..	5-6
Specific Gravity	1.020		1.015 - 1.025
Protein	Nil		Nil
Glucose	Nil		Nil
Ketones	Nil		Nil
Blood	+		Nil
Bilirubin	Nil		Nil
Urobilinogen	Normal		Normal
Nitrite	Negative		

Microscopy

Light Microscopy/Image capture microscopy

Red Blood Cells (RBC)	10-12	/HPF	Nil
White Blood Cells	4-6	/HPF	0.0-5.0
Squamous Epithelial Cells	1-2	/HPF	
Cast	Nil	/LPF	Nil
Crystals	Nil	..	Nil
Bacteria	Nil	/HPF	Nil

Kindly correlate with clinical findings

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Clinical Pathology

Dr. Mrinalini Priyadarshini
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Consultant Pathologist



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Hematology

APTT (Partial Thromboplastin Time), Citrate Plasma*, Sodium Citrate

Date	10/Sep/2021 04:40PM	Unit	Bio Ref Interval
APTT	30.5	Sec	
Photo-Optical-Nephelometry			
Control	28.0	sec	

Interpretation

(Activated Partial Thromboplastin Time; PTT)

APTT is the test which checks the "intrinsic coagulation" pathway and is useful for detecting screening of haemophilia A & B, screening of coagulation inhibitors like lupus anticoagulant.

APTT can also be used to monitor heparin monitoring.

Raised APTT is found in - Liver disease, DIC, heparin treatment, circulating inhibitors (Lupus Anticoagulant), and haemophilia (deficiency of Factor VIII & IX).

Low APTT may be seen in - high concentration Factor VIII and is independent risk factor for increased incidence of thrombus formation.

Advice: - 'APTT mixing study', 'Lupus Anticoagulant test', 'specific Factor(s) assay' may be added on for further evaluation.

Prothrombin Time (with INR), Citrate Plasma*

Date	10/Sep/2021 04:40PM	Unit	Bio Ref Interval
Prothrombin Time (PT)	12.5	Sec	12.1-15.1
Photo-Optical-Nephelometry			
MNPT Value	11.4	Sec	
INR	1.10		

Interpretation

(Syn: - Prothrombin Time)

PT is the test which checks the "extrinsic coagulation" pathway and is useful for detecting coagulation deficiency, liver disease and disseminated intravascular Coagulation (DIC).

PT can also be expressed as International normalized ratio (INR) used for monitoring warfarin therapy.

Raised PT value seen in - factor deficiency (Fibrinogen (I), Prothrombin (II), factor V, VII, X), oral anticoagulation therapy, liver diseases, Vitamin K deficiency and DIC.

Advice: - 'PT mixing study', 'specific factor(s) assay' may be added on for further evaluation.



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Hematology

Platelet Count*, EDTA

Date	10/Sep/2021 04:40PM	Unit	Bio Ref Interval
Platelet Count	252.2	10~9/L	150-410
Electrical Impedance			



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Hematology

CBC (Complete Blood Count)*, Whole Blood EDTA

Date	10/Sep/2021 04:44PM	Unit	Bio Ref Interval
Haemoglobin	12.60	g/dl	12.0 - 15.0
Packed Cell, Volume Calculated	40.3	%	36-46
Total Leucocyte Count (TLC) Electrical Impedance	7.26	10~9/L	4.0-10.0
RBC Count Electrical Impedance	4.75	10~12/L	3.8-4.8
MCV Electrical Impedance	84.9	fL	83-101
MCH Calculated	26.5	pg	27-32
MCHC Calculated	31.3	g/dl	31.5-34.5
Platelet Count Electrical Impedance	252.2	10~9/L	150-410
MPV Calculated	12.02	fL	7.8-11.2
RDW Calculated	14.0	%	11.5-14.5

Differential Cell Count

VCS / Light Microscopy

Neutrophils	75	%	40-80
Lymphocytes	17	%	20-40
Monocytes	07	%	2-10
Eosinophils	01	%	1-6
Basophils	00	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	5.45	10~9/L	2.0-7.0
Absolute Lymphocyte Count	1.2	10~9/L	1.0-3.0
Absolute Monocyte Count	0.51	10~9/L	0.2-1.0
Absolute Eosinophil Count	0.07	10~9/L	0.02-0.5



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Immunoassay

FSH - Follicle Stimulating Hormone,Serum*

Date	10/Sep/2021 04:40PM	Unit	Bio Ref Interval
Follicle Stimulating Hormone CLIA	12.39	mIU/mL	

Ref. Range

Adult Male	1.27 - 19.26
Adult Female :	
Follicular	3.85 - 8.78
Midcycle Peak	4.54 - 22.51
Luteal Phase	1.79 - 5.12
Post Menopausal (>50 Yrs)	16.74 - 113.59

Interpretation

Increased in primary gonadal failure, ovarian or testicular agenesis, Klinefelter's syndrome, Reifenstein's syndrome, castration, alcoholism, menopause, orchitis, gonadotropin-secreting pituitary tumors.

Decreased in anterior hypofunction, hypothalamic disorders, pregnancy, anorexia nervosa, polycystic ovarian disease, hemochromatosis, sickle cell anaemia, severe illness, hyperprolactinemia.

Pooled samples are advisable due to episodic, diurnal and cyclic variations in gonadotropin secretion.



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Immunoassay

LH-Luteinizing Hormone , Serum*

Date	10/Sep/2021 04:38PM	Unit	Bio Ref Interval
Luteinizing Hormone CLIA	11.42	mIU/mL	

Ref Range

LH(Male-Adult)	Reference Range
	1.24-8.62
LH (Female-Adult)	
Follicular	2.12-10.89
Mid Cycle Peak	19.18-103.03
Luteal Phase	1.2-12.86
Post Menopausal (>50 Year)	10.87-58.64

Interpretation

Increased in Primary gonadal dysfunction, polycystic ovarian syndrome (LH/FSH ratio is high in 60% cases), post-menopause, and pituitary adenoma. Decreased in pituitary or hypothalamic impairment, isolated gonadotropic deficiency associated with anosmia or hyposmia (Kallmann's syndrome), anorexia nervosa, isolated LH deficiency ("fertile eunuch"), sever stress, malnutrition, and sever illness. Pooled samples are advisable due to episodic, diurnal and cyclic variations in gonadotropin secretion.



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Laboratory Investigation Report

Patient Name	: Sana Khan	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 37 Y 0 M 0 D /F	OP/IP No	: OP/101330
Max ID/Mobile	: 452256/8752005200	Collection Date/Time	: 10/Sep/2021 04:42PM
Lab ID	: 1266092100242	Receiving Date	: 10/Sep/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 11/Sep/2021
Passport No.	:		

Immunoassay

Prolactin, Serum*

Date	10/Sep/2021 04:38PM	Unit	Bio Ref Interval
Prolactin	59.77	ng/mL	
CLIA			

Ref Range

Males :	2.64 - 13.13
Females :	
Premenopausal	
(<50 years of age):	3.34 - 26.74
Postmenopausal	
(>50 years of age):	2.74 - 19.64

Interpretation

Increased in prolactin-secreting pituitary tumors, amenorrhea and/or galactorrhea, Chiari-Frommel and Argonz Del Cstillo syndromes, various types of hypothalamic-pituitary disease (e.g. sarcoidosis, granulomatous diseases, craniopharyngioma, metastatic cancer, empty sella syndrome), primary hypothyroidism, anorexia nervosa, polycystic ovary syndrome, renal failure, insulin-induced hypoglycemia, chest wall injury, adrenal insufficiency, and pituitary stalk section surgery

Decreased in pituitary apoplexy (Sheehan's Syndrome)



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Immunoassay

TSH,Serum*

Date	10/Sep/2021 04:42PM	Unit	Bio Ref Interval
Thyroid Stimulating Hormone	4.7	µIU/mL	0.34 - 5.6
CLIA			

Interpretation

Parameter	Unit	Premature (28 - 36 Weeks)	Cord Blood (> 37 weeks)	Upto 2 Month	Adult	1st Trimester	2nd Trimester	3rd Trimester
TSH	uIU/ml	0.7 - 27.0	2.3 - 13.2	0.5 - 10	0.38 - 5.33	0.1 - 2.5	0.2 - 3.0	0.3 - 3.0

Increased in primary Hypothyroidism.
Decreased in primary Hyperthyroidism

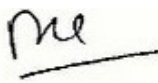
Note : TSH levels are subject to circadian variation, reaching peak levels between 2 – 4 am and at a minimum between 6 – 10 pm. The variation is of the order of 50% - 206 %, hence time of the day has influence on the measured serum TSH concentrations.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Poonam. S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
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**Laboratory Investigation Report**

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Immunoassay

Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist



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Age/Gender	: 37 Y 0 M 0 D /F	OP/IP No	: OP/101330
Max ID/Mobile	: 452256/8752005200	Collection Date/Time	: 10/Sep/2021 04:36PM
Lab ID	: 1266092100242	Receiving Date	: 10/Sep/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 11/Sep/2021
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Serology

Viral Markers (Rapid)

Test Name	Result	Unit	Bio Ref Interval
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Rapid Card Test - HCV*, Serum

Immunochromatography

HCV Card Test Negative

Comment Interpretation

It is only a screening test. All reactive samples should be confirmed by HCV quantitative PCR.
A non reactive result does not exclude the possibility of exposure to or infection with HCV.
Patients with auto immune liver disease may show falsely reactive results

Rapid Card Test - HIV I & II*, Serum

Immunochromatography

Rapid Card HIV I & II Negative

Comment This is only a screening test. All samples detected reactive must be confirmed by using HIV RNA PCR / three different methods.
A non- reactive test result does not exclude the possibility of exposure to an infection with HIV (Window Period) or low viral load.

Advice:

1. HIV by 3 different methods
2. Confirmatory HIV RNA Quantitative PCR.



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Serology

Viral Markers (Rapid)

Test Name	Result	Unit	Bio Ref Interval
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VDRL/RPR*, Serum

RPR(Syphilis)	Non Reactive
Slide Flocculation	

Comment

Interpretation

1. It is a screening test for syphilis which is useful for following the progression of disease and response to therapy. Rising titers are of immense value in confirming the diagnosis.
2. Biological false positive reactions exhibit low titers and are seen in conditions like Viral fevers, Mycoplasma infection, Chlamydia infection, Malaria, Immunizations, Pregnancy, Autoimmune disorders & past history of Treponemal infection.
3. It is advisable to confirm the diagnosis by tests such as TPHA & FTA-ABS.
4. A definitive clinical diagnosis should not be made by result of a single test only, but should be made by taking clinical history and other laboratory findings in to account.

Rapid Card Test - Hepatitis B Surface Antigen*, Serum

Immunochromatography

Rapid Card HBsAg	Negative
------------------	----------

Comment

Interpretation

This is only a screening test.

All reactive samples should be confirmed by 'HBsAg confirmatory test or HBV DNA PCR'.

A non – reactive does not exclude the possibility of exposure to infection with HBV (window period)

False positive results can be obtained due to the presence of other antigens or elevated levels of RF factor.

Advise: Confirmatory test 'HBsAg Confirmatory Quantitative' test followed by 'HBV DNA quantitative PCR'

Kindly correlate with clinical findings

*** End Of Report ***



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Serology

Viral Markers (Rapid)

Test Name	Result	Unit	Bio Ref Interval
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Age/Gender	: 37 Y 0 M 0 D /F	OP/IP No	: OP/101330
Max ID/Mobile	: 452256/8752005200	Collection Date/Time	: 10/Sep/2021 04:40PM
Lab ID	: 1266092100242	Receiving Date	: 11/Sep/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 11/Sep/2021
Passport No.	:		

Immunoassay

Estradiol (E2),Serum

Date	10/Sep/2021 04:40PM	Unit	Bio Ref Interval
Estradiol CLIA	20	pg/mL	

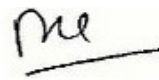
Ref Range

Male	20 - 75
Post - Menopausal (Female)	20 - 88
Estradiol -Total (Non - Pregnant Females)	
Mid Follicular Phase	24 - 114
Mid - Luteal Phase	80 - 273
Periovulatory	62 - 534

Kindly correlate with clinical findings

*** End Of Report ***


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 Manager Quality


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